

SPECIALTY DRUG PRIOR AUTHORIZATION REQUEST

Northgate Benefit Trust | Form NGB-PA-3307 | rev 2026-01 | Policy NGB-SPX-091

SECTION A - PATIENT AND COVERAGE

PATIENT NAME Alice Chen		DATE OF BIRTH 1988-09-14	SEX Female
MEMBER ID NGB771204831	GROUP NUMBER GRP-63007		COVERAGE EFFECTIVE 2026-01-01
PLAN Northgate Benefit Trust · Northgate Open Access EPO		BENEFIT Pharmacy · Specialty tier 4	

SECTION B - PRESCRIBER

PRESCRIBER Elena Vasquez, MD		NPI 1558904712
SPECIALTY Neurology	PRACTICE Harborview Neurology Group	
PHONE (503) 555-0136	FAX (503) 555-0137	TAX ID 93-4418825

SECTION C - REQUESTED THERAPY

REQUESTED PRODUCT Neuvexa (obeltamab) 140 mg/mL		J-CODE J3031
NDC 58291-0140-01	DOSE 140 mg	ROUTE Subcutaneous
FREQUENCY Every 4 weeks	QUANTITY 1 autoinjector per fill	DAYS SUPPLY 28
DIAGNOSIS G43.709 · Chronic migraine without aura, intractable	ICD-10 G43.709	REQUEST TYPE Initial · Standard
SITE OF CARE Self-administered · specialty pharmacy	REQUESTED START 2026-09-10	DURATION REQUESTED 12 months

SECTION D - CLINICAL REVIEW QUESTIONS

Answer every question from the submitted documentation. For each answer, identify the document and page it is supported by.

#	QUESTION	ANSWER	SUPPORTING DOCUMENT / PAGE
1	What specialty drug is being requested?	Neuvexa (obeltamab) 140 mg/mL	Enrollment p1
2	What is the primary diagnosis for the request?	G43.709 Chronic migraine without aura	Chart Note p1
3	How many headache days per month are documented?	17 days per month	Chart Note p1
4	How many preventive agents have been tried?	One — topiramate only	Chart Note p1
5	What preventive therapy was attempted, and for how long?	Topiramate 50 mg daily for 5 weeks	Chart Note p1
6	Why was prior preventive therapy discontinued?	Discontinued for out-of-pocket cost	Chart Note p1
7	Is a step therapy exception documented?	No	Chart Note p1
8	Is the prescriber a neurologist or headache specialist?	Yes — Neurology	Enrollment p1
9	Are step therapy criteria met?	No	Policy p1

SECTION E - ATTESTATION

I certify that the information provided is accurate and that the requested therapy is medically necessary.

SUBMITTER NAME A. Reyes, RN	TITLE Hub clinical reviewer	DATE 2026-08-26
SIGNATURE A. Reyes, RN	HUB CASE HUB-2044	TRACKING NGB-20260826-2044

REVIEWER FLAGS

2 item(s) require attention before this form is considered complete. Step therapy criteria not met.

This form is provided for demonstration purposes and is not the form of any real health plan.

PRESCRIBER ENROLLMENT / START FORM

Neuvexa (obeltamab) 140 mg/mL · Calcitonin gene-related peptide antagonist

SECTION 1 - PATIENT INFORMATION

PATIENT NAME Alice Chen	DATE OF BIRTH 1988-09-14	SEX Female
HUB PATIENT ID PT-4471290	PHONE (503) 555-0191	CONSENT ON FILE Yes · 2026-08-26
HOME ADDRESS 23 Cedar Hollow Way, Portland, OR 97209		

SECTION 2 - INSURANCE

PAYER Northgate Benefit Trust	PLAN Northgate Open Access EPO	
MEMBER ID NGB771204831	GROUP NUMBER GRP-63007	COVERAGE EFFECTIVE 2026-01-01
BENEFIT Pharmacy · Specialty tier 4	SECONDARY COVERAGE None reported	

SECTION 3 - PRESCRIBER

PRESCRIBER Elena Vasquez, MD	NPI 1558904712	TAX ID 93-4418825
SPECIALTY Neurology	PRACTICE Harborview Neurology Group	
PHONE (503) 555-0136	FAX (503) 555-0137	

SECTION 4 - PRESCRIPTION

PRODUCT Neuvexa (obeltamab) 140 mg/mL		J-CODE J3031
NDC 58291-0140-01	STRENGTH 140 mg/mL single-dose autoinjector	
DOSE 140 mg	ROUTE Subcutaneous	FREQUENCY Every 4 weeks
QUANTITY 1 autoinjector per fill	DAYS SUPPLY 28	DURATION REQUESTED 12 months
SITE OF CARE Self-administered · specialty pharmacy		REQUESTED START 2026-09-10
DIAGNOSIS G43.709 · Chronic migraine without aura, intractable		REQUEST TYPE Initial · Standard

SECTION 5 - PRESCRIBER NOTES

New start requested for chronic migraine prophylaxis. Patient has failed topiramate. Please run benefits investigation and submit prior authorization. Patient is highly motivated to begin therapy before the autumn school term.

SECTION 6 - PRESCRIBER SIGNATURE

SIGNATURE Elena Vasquez	TITLE MD	DATE SIGNED 2026-08-26
-----------------------------------	--------------------	----------------------------------

This document is synthetic and was produced for software demonstration purposes. It does not describe a real patient, prescriber, payer, or product.

NEUROLOGY PROGRESS NOTE

Follow-up visit · Established patient

Patient Alice Chen · 37F · DOB 1988-09-14
Patient ID PT-4471290
Primary diagnosis G43.709 · Chronic migraine without aura, intractable

SUBJECTIVE

The patient returns for follow-up of chronic migraine. She has maintained a headache diary for four months, averaging 17 headache days per month, of which 9 to 11 are migrainous with photophobia, phonophobia, and nausea. Attacks last 6 to 20 hours. She uses sumatriptan 100 mg for acute treatment on approximately 8 days per month with partial relief. She was started on topiramate 25 mg daily on 2026-05-12, titrated to 50 mg daily on 2026-05-26, and stopped it on 2026-06-16 after five weeks. She stopped because her out-of-pocket cost rose after a formulary change, not because of side effects; she reports she tolerated it without paraesthesia or cognitive complaints. She has not tried a beta blocker, a tricyclic, an antihypertensive, or onabotulinumtoxinA. She is not pregnant and is using contraception. There is no history of an adverse drug reaction requiring hospitalisation.

OBJECTIVE

Alert and oriented. Cranial nerves II through XII intact. Motor 5 out of 5 throughout. Sensation intact. Reflexes 2 plus and symmetric. Gait normal. Fundoscopic examination normal with sharp disc margins. No cervical or occipital tenderness. Blood pressure 118 over 74.

ASSESSMENT

Chronic migraine without aura, intractable, G43.709, at 17 headache days per month by diary over four months. One preventive agent trialed: topiramate, 50 mg daily, five weeks, discontinued for out-of-pocket cost rather than intolerance or lack of efficacy. No beta blocker, tricyclic, or other preventive class has been attempted. No contraindication to remaining preventive classes is present. No pregnancy, no prior hospitalising adverse drug reaction. Not currently on any CGRP antagonist. Acute medication use is 8 days per month, below the medication overuse threshold.

PLAN

Requested Neuvexa (obeltamab) 140 mg subcutaneously every four weeks and enrolled the patient through ArvelisConnect. I recognise the plan may require a second preventive trial. If prior authorization is not approved, initiate propranolol LA 80 mg daily and titrate to 160 mg over four weeks, continue the headache diary, and reassess at eight weeks for resubmission. Continue sumatriptan for acute attacks and monitor acute medication days. Return in eight weeks.

ELECTRONICALLY SIGNED BY

Elena Vasquez, MD

DATE / TIME

2026-08-26 14:02

This document is synthetic and was produced for software demonstration purposes. It does not describe a real patient, prescriber, payer, or product.

MEDICAL POLICY NGB-SPX-091

Northgate Benefit Trust | CGRP Antagonists for Migraine Prophylaxis | Effective 2026-01-01

SCOPE

This policy establishes the medical necessity criteria applied by Northgate Benefit Trust to requests for cgrp antagonists for migraine prophylaxis for commercial plan members. It applies to requests submitted directly by a prescriber and to requests submitted by an authorised manufacturer hub agent on the prescriber's behalf.

DRUGS REQUIRING PRIOR AUTHORIZATION

Prior authorization is required for products billed under the following codes: J3031. J3031 corresponds to obeltamab, marketed as Neuvexa, 140 mg/ml single-dose autoinjector.

DRUGS NOT REQUIRING PRIOR AUTHORIZATION

Acute migraine abortive therapies, including triptans and non-steroidal anti-inflammatory drugs, do not require prior authorization under this plan.

MEDICAL NECESSITY CRITERIA

Neuvexa is considered medically necessary when all of the following criteria are met:

1. The patient has a diagnosis of chronic migraine, defined as fifteen (15) or more headache days per month for at least three months, documented in a headache diary or clinical note.
2. The patient is 18 years of age or older.
3. The patient has completed a trial of at least two (2) migraine preventive agents from separate pharmacologic classes, each sustained for a minimum of eight (8) weeks at a therapeutic dose, without adequate response or with documented intolerance.
4. The drug is prescribed by, or in documented consultation with, a neurologist or headache specialist.
5. The patient is not concurrently receiving another calcitonin gene-related peptide antagonist.

EXCEPTIONS

The two-agent step therapy requirement is reduced to one agent where the patient has a documented contraindication to all remaining classes, is pregnant or planning pregnancy, or has a documented history of an adverse drug reaction requiring hospitalisation. Discontinuation for cost, convenience, or patient preference does not satisfy the step therapy requirement.

DOCUMENTATION REQUIREMENTS

Submitted requests must include the signed prescriber enrollment or start form and a clinical note documenting headache frequency, each preventive agent tried with its dose and duration, and the reason each was discontinued.

REVIEW TIMEFRAMES

Standard requests are adjudicated within five (5) business days of receipt of a complete submission. Expedited requests are adjudicated within seventy-two (72) hours. An incomplete submission is returned with a written statement of the missing element and the review clock restarts on receipt of the complete packet.

CODING

CODE	TYPE	DESCRIPTION
J3031	HCPCS	Obeltamab, per billing unit
58291-0140-01	NDC	Neuvexa 140 mg/mL single-dose autoinjector
G43.709	ICD-10-CM	Chronic migraine without aura, intractable

This document is synthetic and was produced for software demonstration purposes. It does not describe a real patient, prescriber, payer, or product.